SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Serplulimab, carboplatin and etoposide

First-line extensive-stage small-cell lung cancer · NCCP 00908 · version 1b · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
This treatment aims to control extensive-stage small-cell lung cancer, relieve symptoms and help people live longer. The balance of benefit, side effects and daily life is individual; it is generally not offered with an expectation of cure.

Schedule
Induction lasts four planned 21-day cycles unless treatment needs to stop earlier. On Day 1 you receive serplulimab, carboplatin and etoposide by drip. With the standard IV schedule, etoposide is repeated on Days 2 and 3. After induction, carboplatin and etoposide stop. Serplulimab alone can continue every three weeks while it is helping and its risks remain acceptable. There is no fixed universal number of maintenance cycles in this NCCP regimen. Your team may instead prescribe etoposide by drip on Day 1 and capsules twice daily on Days 2 and 3. This is a separate alternative, not extra treatment. Oral and IV doses are not interchangeable milligram for milligram. Confirm which plan you have; take the capsules on an empty stomach as prescribed.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Serplulimab
IMMUNOTHERAPY

- ☐ Immune inflammation of lungs, bowel, liver, kidneys, skin and hormone glands
- ☐ Immune effects can also involve the heart, pancreas, nervous system or blood cells
- ☐ Severe infusion reactions
- ☐ Some hormone effects need lasting replacement treatment; immune effects can begin after the final dose

Carboplatin
CHEMOTHERAPY

- ☐ Low blood counts, infection or bleeding
- ☐ Nausea and vomiting
- ☐ Allergic reactions
- ☐ Kidney, hearing and nerve effects

Etoposide
CHEMOTHERAPY

- ☐ Low blood counts, infection and bleeding
- ☐ Hair loss and mouth or bowel symptoms
- ☐ Infusion reactions and low blood pressure
- ☐ A delayed risk of treatment-related leukaemia

Discuss expected benefit, alternatives including symptom-focused care, infection and bleeding risks, immune organ inflammation and possible lasting hormone replacement, infusion reactions, fertility/contraception, delayed effects and the different IV/oral etoposide schedules. Your priorities and the option to reconsider treatment belong in this conversation.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00908 v1b.

SACTCheck v0.84.23
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